

AI in Clinical Decision-Making and Pattern Recognition

Emerging Technologies, Market Dynamics, and Strategic Investment Directions

Vidhyalakshmi Harikrishnan

PhD Candidate, Artificial Intelligence & Human-Computer Interaction

University of Cumberlands

June 2026

Conceptual Overview and Current Trends

CONCEPTUAL OVERVIEW

- AI-driven decision support synthesizes patient data, clinical evidence, and context — CNNs and transformers reach radiologist-level accuracy on imaging, while LLMs and NLP read unstructured clinical text.
- The **Human-AI-Human paradigm** keeps final decision authority with physicians; combined performance outperforms either working independently.

CURRENT MARKET TRENDS

- AI medical imaging is projected to grow from **\$4.5B (2024) to \$14.25B (2032)** at 14.6% CAGR; broader AI-in-diagnostics from \$1.35B to \$8.07B by 2033 at 21.97% CAGR.
- **Deep learning drives 57.67%** of AI imaging revenue, led by CNNs' performance in image recognition.
- OEMs — **GE Healthcare, Siemens Healthineers, Philips** — embed AI decision support directly into imaging hardware.
- Cloud vendors — **Aidoc, Viz.ai, Qure.ai** — deploy disease-specific algorithms for real-time stroke and pulmonary-embolism triage.
- Regulatory maturation (FDA, CE, PMDA, Medicare pathways) accelerates adoption; **clinician trust**, shaped by explainability, remains the key variable.

Opportunities, Threats and Future Direction

Opportunities	Threats	Future Direction — Cotiviti
■ Diagnostic equity and precision oncology — AI-assisted diagnosis of TB, cataracts, and cancers in Tamil Nadu government hospitals. ■ Eliminated a 54,000-scan backlog at Gold Coast University Hospital — signaling high growth in LMIC markets. ■ Prevention-first claims integrity — anomaly detection flags billing irregularities, duplicates, and upcoding before payment.	■ Algorithmic bias — models trained on historical claims inherit the coding and billing biases of past populations. ■ False positives create provider abrasion; false negatives miss fraud in underrepresented billing categories. ■ Black-box opacity creates auditability and regulatory exposure under transparency mandates. ■ Data privacy risk and regulatory fragmentation across jurisdictions.	■ Deploy a generative-AI, pre-claim pattern recognition layer on the Edifecs interoperability infrastructure. ■ LLM models detect anomalous billing and predict high-risk coding at documentation, surfacing explainable guidance before adjudication. ■ Compete in the pre-claim market vs. Codoxo's Point Zero — 100% retention, \$66 PMPY across 80M lives. ■ Embed Explainable AI (SHAP, counterfactuals) to meet G7 and WHO mandates — turning data scale into a generative-AI moat.

The Principles We Used

Principle	What it means	How it helps healthcare analysis
Explainable AI (XAI)	Every flag comes with a numbered reasoning trace, not just a score	Clinicians and auditors can act on a decision instead of trusting a black box
Chain Reasoning	The system reasons in sequential steps, like a human investigator	Each step builds to a transparent, defensible final verdict
SHAP Feature Attribution	Game-theory scores show how much each input drove a decision	Pinpoints which factors mattered most, and by how much
Supervised Pattern Recognition	Learns what normal billing and claims look like, flags deviations	The core engine behind FWA detection and anomaly scoring
Unsupervised Clustering	Finds natural groupings in data without being told what to seek	Surfaced five distinct member risk populations on its own
Anomaly Detection (Isolation Forest)	Measures how easily a data point can be isolated from peers	Easy-to-isolate billing patterns are flagged as unusual
Pre-claim Intervention	Catch a problem before it happens, not after payment	Shifts the model from reactive review to proactive prevention
Risk Stratification	Ranks members and claims by risk level	Directs time and resources to the highest-priority cases first

Functions the Tool Performs

Function	What it does
Pre-claim Clinical Monitoring	Watches prior auths, prescriptions, and lab orders before a claim exists, flagging issues at the point of care
Claim Pattern Analysis	Runs a claim through the full AI engine and returns a risk-scored verdict with complete reasoning
FWA Detection	Combines code mismatches, frequency outliers, and auth gaps to surface fraud, waste, and abuse
Denial Risk Prediction	Estimates the probability a claim will be denied from auth status, claim amount, and member profile
Procedure-Diagnosis Alignment	Checks the billed procedure fits the diagnosis on record — a mismatch is a top FWA signal
Provider Anomaly Scoring	Benchmarks billing against peers with an isolation-forest score for outlier behavior
Member Risk Clustering	Segments a population into distinct risk cohorts for targeted intervention
Care Gap Inference	Infers which conditions are likely unaddressed or underdocumented from claims history
HCC Coding Accuracy Estimation	Estimates how accurately member health complexity is captured in coding, which drives reimbursement
Readmission Risk Prediction	Estimates 30-day readmission risk for discharged members to enable proactive outreach

Technologies Behind the Prototype

Technology used	What it does in the prototype
Rule-based Scoring Engine	Encodes clinical and regulatory guidelines as fast, transparent, auditable logic rules
Isolation Forest Algorithm	Detects anomalies by measuring how quickly a data point is isolated — outliers separate fastest
k-means Clustering	Groups members into five risk cohorts by minimizing within-cluster distance
SHAP Value Computation	Attributes each prediction to individual input features for interpretable reasoning
Probability Scoring Models	Translate multiple signals into a 0–1 probability for denial, FWA, and readmission risk
Event Stream Simulation	Mimics real-time clinical events arriving from an EHR or health information exchange
Vanilla JavaScript (ES5)	Runs all interactive logic with no libraries — works in any browser, no install
HTML and CSS	Builds the entire interface in standard web tech — portable, lightweight, no build tools

PROTOTYPE

The Tool Interface

Healthcare Pattern Recognition

XAI DEMO — PRE-CLAIM · TREATMENT · PAYMENT · OPERATIONS

Pre-claim

Claim Analysis

Anomaly

Risk Clusters

● Live clinical events

All (5)

Flagged (2)

Warning (2)

Approved (1)

PRIOR AUTH REQUEST

Knee replacement — bilateral

Member #M-4821 · Dr. Ortega · Northside Surgical

High risk

PRESCRIPTION ORDER

Opioid combination — high MME

Member #M-2290 · Dr. Patel · CVS #1142

Urgent flag

TREATMENT ORDER

MRI lumbar spine — acute back pain

Member #M-7734 · Dr. Kim · Metro Imaging

Review

LAB ORDER

Metabolic panel + lipid screen

Member #M-1056 · Dr. Nguyen · Quest Diagnostics

Approved

PRIOR AUTH REQUEST

Inpatient psychiatric admission

Member #M-3367 · Dr. Osei · Lakeview Behavioral

Review

ACCESS THE LIVE TOOL

Explore the working prototype — pre-claim monitoring, FWA detection, and explainable risk scoring in the browser.

TOOL LINK

<https://vidhikrish.github.io/Healthcare-pattern-recognition/>